

CLINICAL DISCHARGE SUMMARY

Patient Name: Aarav Sharma (Synthetic)	UHID / IP No: APX-2026-90412
Age / Gender: 29 Years / Male	Consultant: Dr. S. K. Sen, MD, DM (Pulmonology)
Date of Admission: 02-Oct-2026	Date of Discharge: 05-Oct-2026

Final Diagnosis

Acute Exacerbation of Bronchial Asthma (Moderate Persistent)

Brief Hospital Course

Patient presented to Emergency with acute onset breathlessness, wheezing, and chest tightness. SpO2 was 91% on room air with PEFr 220 L/min. Treated with nebulized Salbutamol and Ipratropium bromide, intravenous Hydrocortisone, and supplemental oxygen. Bronchospasm resolved gradually. Chest X-ray showed hyperinflated lung fields without parenchymal consolidation. Patient is hemodynamically stable, afebrile, SpO2 98% on room air, and ambulating comfortably.

Discharge Medications

- Inhaler Budesonide + Formoterol (200/6 mcg) - 2 puffs twice daily via spacer with mouth rinsing after use
- Inhaler Levosalbutamol (50 mcg) - 2 puffs SOS for acute breathlessness / relief
- Tablet Prednisolone 20 mg once daily orally in the morning with breakfast for 3 days, then stop
- Tablet Montelukast 10 mg once daily orally at bedtime for 30 days

Follow-up Instructions

Review in Pulmonology OPD with Dr. S. K. Sen on 12-Oct-2026 at 10:30 AM with Peak Flow diary. Re-evaluate spirometry in 4 weeks.

Warning Signs and Emergency Advice

- Severe worsening breathlessness not relieved after 2-4 puffs of SOS reliever inhaler
- Difficulty speaking complete sentences in one breath or blue discoloration of lips/fingertips
- Peak Expiratory Flow Rate (PEFR) dropping below 60% of personal best
- In case of any emergency red-flag symptoms, report immediately to the 24x7 Emergency Casualty Department

General Advice & Lifestyle

- Strict avoidance of dust, smoke, incense sticks (agarbatti), and mosquito coil fumes
- Maintain adequate hydration with warm fluids; annual influenza vaccination recommended